Information For the Use of
Glimepiride (2mg) + Metformin (850mg) SR Bilayered Tablet

Gimtype M2/850 Tablet

SIPAGETIN DM 500 TABLETS

Composition

Each film-coated tablet contains:

Dapagliflozin 10 mg
Metformin Hydrochloride 500 mg
(as Extended Release)
Sitagliptin 100 mg
Excipients … q.s.

Pharmacological Action

Dapagliflozin is a selective sodium-glucose cotransporter-2 (SGLT2) inhibitor. It reduces renal glucose reabsorption, resulting in increased urinary glucose excretion and reduction of blood glucose levels. It also promotes mild natriuresis and osmotic diuresis.

Metformin Hydrochloride is a biguanide antihyperglycaemic agent. It primarily reduces hepatic glucose production and improves peripheral insulin sensitivity, thereby increasing glucose uptake and utilization.

Sitagliptin is a dipeptidyl peptidase-4 (DPP-4) inhibitor. It increases the levels of active incretin hormones, resulting in glucose-dependent enhancement of insulin release and reduction of glucagon secretion.

The combination provides complementary glucose-lowering effects through **increased urinary glucose excretion, reduced hepatic glucose production, improved insulin sensitivity and glucose-dependent enhancement of insulin secretion**.

Indications

Sipagetin DM 500 Tablets are indicated as an **adjunct to diet and exercise to improve glycaemic control in adults with Type 2 Diabetes Mellitus** when combination therapy is considered appropriate.

CDSCO records include dapagliflozin + sitagliptin + extended-release metformin combinations with the indication of improving glycaemic control in adults with Type 2 Diabetes Mellitus as an adjunct to diet and exercise.

Dosage and Administration

Adults:

The dosage should be individualized according to the patient's current treatment regimen, glycaemic response, renal function and tolerability.

For the **10 mg dapagliflozin + 100 mg sitagliptin + 500 mg extended-release metformin** strength, the tablet is generally administered **once daily with a meal**, as prescribed by the physician.

- Take the tablet with food to reduce gastrointestinal adverse effects associated with metformin.
- Swallow the extended-release tablet whole with sufficient water.
- **Do not crush, split or chew** the tablet.
- Monitor fasting blood glucose and HbA1c regularly.
- Renal function should be assessed before initiation and periodically thereafter.

Dose selection should take into account the patient's renal function and previous antihyperglycaemic therapy.

Precautions

- Assess **renal function** before starting treatment and periodically during therapy.
- Dapagliflozin may cause increased urination, volume depletion and hypotension, particularly in elderly patients and those receiving diuretics.
- Maintain adequate hydration, particularly during conditions associated with fluid loss.
- Monitor for symptoms of **genital mycotic infections and urinary tract infections**.
- Rarely, SGLT2 inhibitors may cause **euglycaemic diabetic ketoacidosis**. Consider this possibility in patients with nausea, vomiting, abdominal pain, malaise or difficulty breathing even when blood glucose is not markedly elevated.
- Metformin should be temporarily withheld during significant dehydration, severe acute illness or other conditions associated with increased risk of lactic acidosis.
- Assess renal function before iodinated contrast procedures and temporarily discontinue metformin-containing treatment when clinically indicated.
- Monitor for persistent or severe abdominal pain because pancreatitis has been reported with DPP-4 inhibitors.
- Hypoglycaemia is generally uncommon when used without insulin or insulin secretagogues, but the risk increases when used with these agents.
- Consider monitoring vitamin B12 levels during prolonged metformin treatment, particularly in patients with risk factors for deficiency.

Contraindications

Sipagetin DM 500 Tablets are contraindicated in:

- Known hypersensitivity to **dapagliflozin, sitagliptin, metformin or any component** of the formulation.
- Severe renal impairment or patients for whom metformin/dapagliflozin therapy is contraindicated because of renal function.
- **Acute or chronic metabolic acidosis**, including diabetic ketoacidosis.
- Severe hepatic impairment where metformin therapy is inappropriate.
- Patients with Type 1 Diabetes Mellitus.

Adverse Reactions

Common/Possible:

- Nausea
- Diarrhoea
- Abdominal discomfort
- Dyspepsia
- Increased urination
- Genital fungal infections
- Urinary tract infections
- Headache
- Nasopharyngitis

Dapagliflozin-related:

- Volume depletion
- Dizziness
- Hypotension
- Increased thirst

Sitagliptin-related:

- Upper respiratory tract symptoms
- Headache
- Joint pain

Rare/Serious:

- Diabetic ketoacidosis
- Acute kidney injury or deterioration of renal function
- Severe hypersensitivity reactions
- Pancreatitis
- Metformin-associated lactic acidosis
- Severe genital/perineal infections

Drug Interactions

Insulin and insulin secretagogues:
Concomitant use may increase the risk of hypoglycaemia. Dose adjustment may be required.

Diuretics:
May increase the risk of volume depletion and hypotension with dapagliflozin.

Medicines affecting renal function:
Drugs that reduce renal function may increase the risk associated with metformin and should be reviewed.

Iodinated contrast media:
Metformin may need to be temporarily discontinued around contrast procedures depending on renal function and the clinical situation.

Cationic medicines:
Medicines eliminated by renal tubular secretion may potentially affect metformin exposure.

Strong enzyme/transporter inhibitors or inducers:
Potential interactions with sitagliptin should be reviewed according to concomitant therapy and prescribing information.

Patients should inform their healthcare professional about all prescription medicines, OTC medicines, herbal products and supplements being used.

Pregnancy and Lactation

Sipagetin DM 500 Tablets are **not generally recommended during pregnancy**. Dapagliflozin is generally avoided during pregnancy, particularly during the second and third trimesters, because of potential effects on the developing fetal kidney.

If pregnancy occurs during treatment, the patient should contact the treating physician promptly.

Use during breastfeeding is generally **not recommended**, particularly because of the dapagliflozin component. The treating physician should determine an appropriate alternative when required.

Storage

Store in a **cool, dry place**, protected from excessive heat, moisture and direct sunlight.

Keep the tablets in their original packaging and **out of reach of children**.

Do not use after the expiry date.

Note

For use under medical supervision.

Sipagetin DM 500 Tablets should be used as an adjunct to appropriate **diet and exercise**. Regular monitoring of blood glucose, HbA1c and renal function is recommended.

Patients should maintain adequate hydration and promptly report symptoms suggestive of ketoacidosis, severe infection, pancreatitis or significant dehydration.

Important: The final printed literature should follow the **approved product-specific Indian prescribing information**, particularly for renal-function thresholds, dosage adjustments, contraindications and approved indications.

Manufactured in India for:

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LIFECARE

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